

Name : Mr. SHIVAM DIXIT	Age : 32 Years
Lab No. : 473192096	Gender : Male
Ref By : Self	Reported : 2/8/2025 7:06:16PM
Collected : 2/8/2025 3:44:00PM	Report Status : Final
A/c Status : P	Processed at : LPL - Bhopal Lab II
Collected at : DHEERAJ AGRAWAL (BHOPAL CC 62) HIG DH 19 AYODHYA EXTENTION AYODHYA BYPASS BHOPAL 462021 Bhopal	Plot No.05, Mandakini Housing Society, Near Apurti Shopping Mall, Kolar Main Road, Bhopal, M.P. - 462042

Test Report

Test Name	Results	Units	Bio. Ref. Interval
HbA1c (GLYCOSYLATED HEMOGLOBIN), BLOOD (HPLC, NGSP certified)			
HbA1c	5.6	%	4.00 - 5.60
Estimated average glucose (eAG)	114	mg/dL	

Interpretation

HbA1c result is suggestive of non diabetic adults (≥ 18 years)/ well controlled Diabetes in a known Diabetic

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults ≥ 18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	≥ 6.5	< 7.0

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.

FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS
Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIPID SCREEN, SERUM (CHO-POD)			
Cholesterol, Total	218.60	mg/dL	<200.00
Triglycerides	251.00	mg/dL	<150.00
HDL Cholesterol	43.96	mg/dL	>40.00
LDL Cholesterol, Calculated	124.44	mg/dL	<100.00
VLDL Cholesterol, Calculated	50.20	mg/dL	<30.00
Non-HDL Cholesterol	175	mg/dL	<130

Advice: Direct LDL Cholesterol (B129)

Please note, Calculated LDL Cholesterol may be underestimated in the setting of high triglyceride levels, which could result in under treatment of high-risk patients.

Note

- Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
- Lipid Association of India (LAI) recommends screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors
- Triglycerides levels >150 mg/dL in fasting or >175 mg/dL in non-fasting are considered risk modifier for ASCVD risk.

Treatment Goals for Lipid lowering therapy (as per Lipid Association of India 2023)

ASCVD RISK CATEGORY	TREATMENT GOAL	
	LDL-C in mg/dL (Primary target)	NON HDL-C in mg/dL (Co-Primary target)
Low	<100	<130
Moderate	<100	<130
High	<70	<100
Very High	<50	<80
Extreme (A)	<50(<30 optional)	<80(<60 optional)
Extreme (B)	<30	<60



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Test Report

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ASCVD Risk Stratification & Treatment goals in Indian population

Indians are at very high risk of developing ASCVD, they usually get the disease at an early age, have a more severe form of the disease and have poorer outcome as compared to the western populations. Many individuals remain asymptomatic before they get heart attack, ASCVD risk helps to identify high risk individuals even when there is no symptom related to heart disease. Risk stratification is important to guide lipid lowering therapy and to identify treatment goals.

CSI Clinical Practice guidelines (2024) recommends in the absence of formal risk calculator for Indian population, only risk factors can be used for risk assessment. Standard Risk factors are:

- Smoking/tobacco use
- Hypertension
- Diabetes
- Family h/o Premature CAD (Men <55 years and women <60 years)

Risk Assessment*

Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
No standard risk factor	Presence of any one standard risk factor	• Presence of 2 or more standard risk factors with no manifest ASCVD • DM with 1 or more risk factor • Heterozygous Familial Hypercholesterolemia (HeFH) with no risk factor • Hypertension with one or more risk factor or with Target organ damage (TOD) • CKD- eGFR 30-59 ml/min	• ASCVD- CAD/PVD/CeVD • Imaging->50% lesion in any two major vessels • DM>20 years or multiple risk factors,TOD • HeFH -with ASCVD or RF • CKD- eGFR <30 ml/min	• ASCVD with recurrent vascular events. • ASCVD with HeFH & High Lp(a)

* A more formal risk assessment may be used by clinicians according to their personal preferences and familiarity with the risk scores



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	Bhopal		

Test Report

Test Name	Results	Units	Bio. Ref. Interval
RHEUMATOID FACTOR (RA), SERUM (Immunoturbidimetry)	<10.00	IU/mL	<14.00



Dr. Swapnil Gupta
MD, Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd



Dr Kiran Bhargava Pathak
MD, Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd

-----End of report-----



IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory .
•Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician .•Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner /Doctor. •The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

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